Last update: January 2021

Communication and advice in gynecological consultations

Author: Cécile Longoni

Co-Author(s): Prof. Dr. med. Johannes Bitzer, Dr. med. Judith Alder

Introduction

In obstetrical and gynecological consultations or visits, the aim is to record the patient's current issues and symptoms and establish the relevant diagnosis. It is important to differentiate between two elements or levels to which parts of the obstetrical/gynecological consultation or visit can be assigned: the biomedical level with the underlying biomedical model, and the biopsychosocial level with the underlying biopsychosocial model.

Biomedical model

- Diagnostic procedures and standards
- Therapeutic procedures and standards

Biopsychosocial model

- Relationship
- Communication

Biomedical model

On the biomedical level, during the consultation the doctor pays attention to the structure and function of the genital organs with the aim of diagnosing and treating obstetrical/gynecological diseases as described in textbooks. This part of the consultation is based on the biomedical model:

- Symptoms are the consequence of objective structural changes.
- These structural changes can be identified using standardized diagnostic methods.
- They can be treated, cured or undone using standardized treatment processes.

Biopsychosocial model

The biomedical element of the consultation depends on the personal encounter between the patient and doctor. The patient as an individual has specific thoughts, feelings and behaviors, which they experience in a specific context. By communicating with the patient, the doctor can develop a relationship with them. This part of the consultation is based on the biopsychosocial model. [B07][B25]

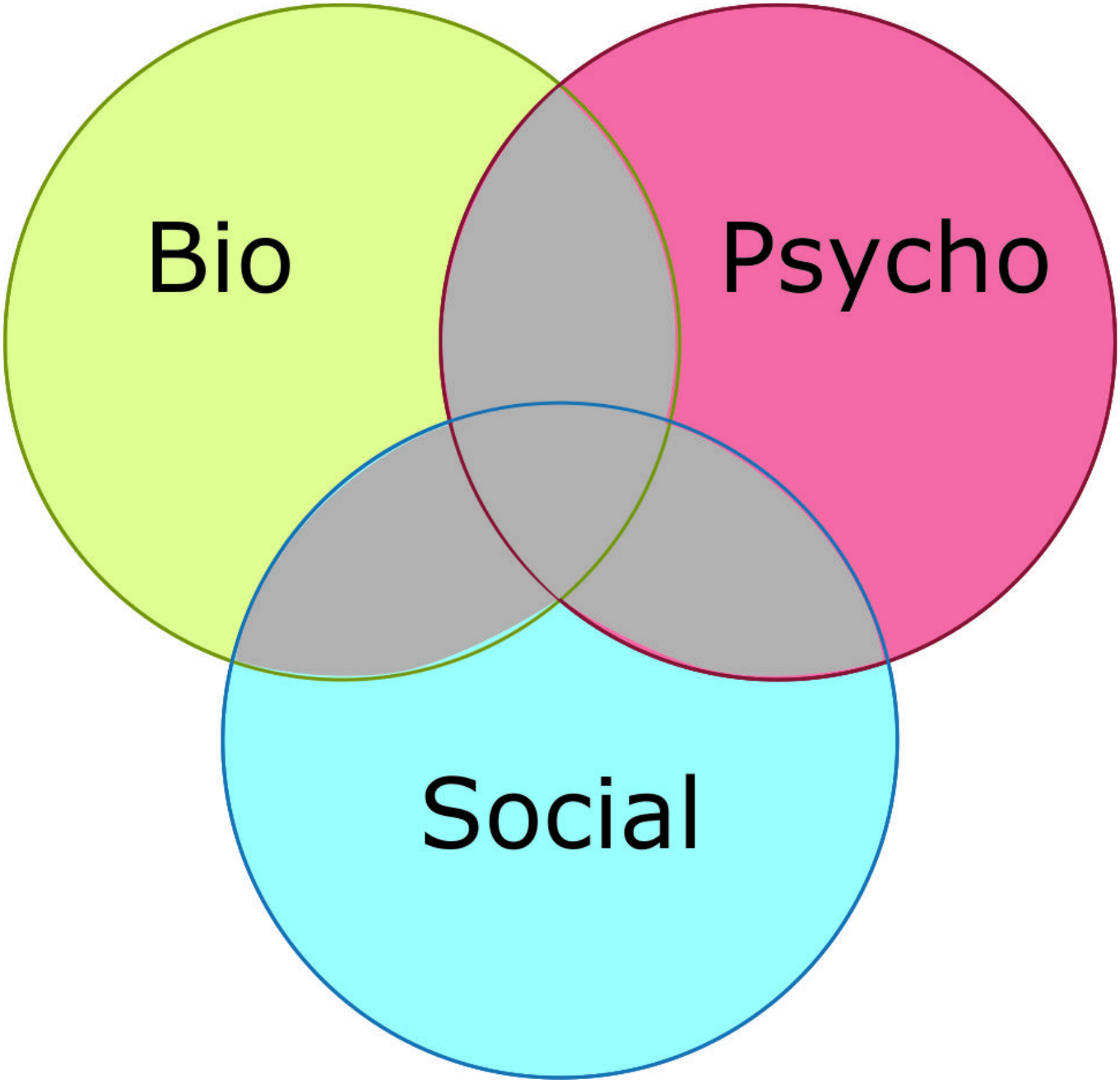

Figure 1: Biopsychosocial model.

- Symptoms are the expression of individual suffering, brought on by a combination of biological, psychological and social factors.
- The biopsychosocial diagnostic aims to record this individual suffering and understand the determining factors.
- Treatment interventions include decision making and collaboration with the patient. The patient is an active partner in the treatment.

Communication styles

Doctor-focused communication style

In this communication style, the doctor clearly takes the lead in the conversation and also determines the agenda. Doctor-focused communication is characterized by its unilateral, vertical nature. The doctor speaks almost exclusively. In this hierarchical doctor-patient dialog, the patient takes on the role of an uninformed, passive recipient. With this top-down communication, the flow of communication goes down from the doctor. The patient is quickly interrupted, the doctor mainly asks closed questions (“yes”, “no” or short answers only). The aim is to provide information quickly and efficiently or rapidly reach a diagnosis. There is usually no room for emotion.
The information acquired is well-structured and organized, and helps to quickly define the problem and find solutions using an evidence-based scientific approach. This communication model is suitable for emergency wards, clearly physical diseases or specific issues.

Patient-focused communication style

A different approach is offered by patient-focused communication, which involves not only the disease but also the patient in his social context. In this communication style, the doctor must take into account two further factors in addition to the factual level:

- Factual level
- Emotional level
- Relationship level

Table 1: Elements of patient-focused communication

Empathy	Empathy, not sympathy How does the patient see themselves and the world? What is important for them, or unimportant?
Respect and value	The patient lives in a different context and is different - but at the same time similar as a human being. Be aware of this difference and similarity. Respect it and recognize it as having value.

Authenticity	Recognize strengths, weaknesses and limits, and be able to communicate/express them if necessary Be authentic, don't pretend or play a role

Relationship level

Studies show that the relationship level is by far the most significant aspect of communication. It offers information on how factual statements should be understood, as the other party pays attention to how something is expressed in terms of body language, gestures and voice. [B25]

The following are potential strategies to help with relationship building:

- Joining (starting and developing the conversation): Building a relationship starts from the very first greeting. Give your name and role, and make a connection with the patient, e.g. "You must have been waiting a long time...", "I see you've come a long way today."
- Clarify what the consultation is about: Provide the patient with space to formulate their own ideas or questions, e.g. "How can I help you?", "Is there a specific issue we should cover?"
- Content and purpose of the consultation: Establish why the patient is at the consultation (doctor's information), and the time limits.
- Ask for feedback: Determine whether the planned consultation matches the patient's needs "Is that okay for you?"
- Structure the conversation: Be transparent about limitations in terms of time and content, e.g. in case a question is not/cannot be covered. Define the topics of the conversation: what will/can be discussed in the consultation. Announce transitions "I think I've now understood what your concern is. Now I'd like to ask you some general questions about your history."
- Summarize what you've said and heard: The questions and concerns discussed during the consultation often entail a great deal of information in a short time for patients. Summarizing and asking for confirmation makes it possible to determine what the patient has already understood, and what must still be clarified. "Can I summarize what I've understood so far? Please correct me if anything is incorrect."

Emotional level

Emotions are the closest representation of the patient's individual reality and their perception. The emotional brain is the quickest and most intuitive. Emotions are expressed not only with words, but also with body language. For the patient, suppressing their emotions means they don't feel understood. The NURS schema can be helpful when dealing with emotions. Of course, this is not only about the patient's emotions, but also the doctor's.

NURS schema			
Name	**U**nderstand	**R**espect	**S**upport

Figure 2: NURS schema.

Name
Identify individual emotional reactions: What feelings does the patient trigger in me? What sensations am I noticing?

Understand
Perceive the patient's emotions: How are they? What feelings are being expressed by their behavior? It can be helpful to pause to give the patient time and space to express themselves.

Respect
Respect the patient's feelings: Express respect and acceptance specifically, for example in the form of a question: "It looks like this information is a lot for you right now, I'd like to give you a moment - please let me know when you're ready to continue." Use the patient's own words.

Support
Support talking about emotions: give the patient time, silence can be helpful; give the patient space for corrections.

Content/subject level

The content level is where “facts” are transmitted. This almost always refers to the subject matter content of a statement.
This part of the consultation should provide the patient with relevant information on their condition. It is important to note that the doctor should remain in constant contact with the patient. Information transmission should be an exchange, not a one-way street. The patient’s experiences and circumstances should be taken into account. In addition, the patient’s right not to want to know everything should be respected.

Information transmission as a one-way street

- The doctor communicates their knowledge in their words
- They ensure completeness and scientific accuracy
- Lecture-style
- Top-down

Information exchange

- Meet the patient's needs and questions
- The doctor creates a shared platform with the patient
- The doctor aims for “adapted” information
- The doctor encourages the patient to ask questions and provides further explanation

Elicit-provide principle

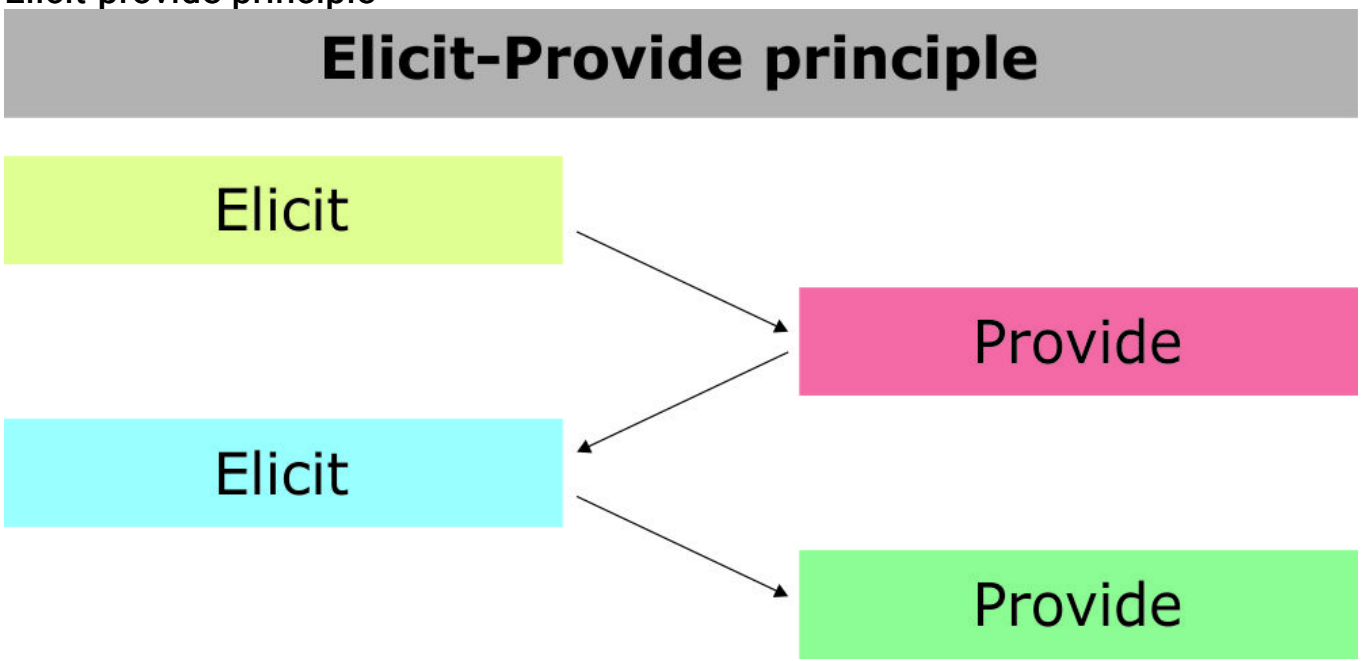

Figure 3: Elicit-Provide principle.

The Elicit-Provide principle is a useful schema for designing information transmission as an exchange. The patient should be invited to take an active role in the conversation, and express their own thoughts, questions and concerns. The doctor can then address these needs and, if relevant, provide further information.

Elicit: What information does the other person need?

- What and how much does the patient want to know?

Provide: Based on the previous step, give the patient the selected basic information.

- Emphasize important details
- Break information down into small units
- Summarize important aspects of the conversation
- Give the patient opportunities to ask questions (or encourage them to ask questions)

Elicit: The current level of understanding should be determined, for example by asking whether the information has been understood or if something was too quick, too unclear or incomprehensible. This also includes questions on the emotional processing of information such as:

- "How do you feel about this information?"
- "Does what I've just said make you feel anxious or give you more security?"
- "Have any new questions come up?"

Provide: Based on the previous step, new needs, doubts or questions can be covered.

When transmitting scientific information, it is helpful to make use of different sensory channels, for example images, brochures, writing or drawing. The response to different sensory channels increases understanding and retention of information.

Summary of patient-focused communication style

Patient-focused communication entails three levels: the subject level, the emotional level and the relationship level. At the subject level, important information should be transmitted to the patient as an exchange, and the patient's questions and needs acknowledged. The Elicit-Provide principle helps to avoid prefabricated questions in favor of an actual dialog with the patient. Patients are involved as active participants in the decision-making process. At the emotional level, the emotions of both the patient and the doctor are expressed, acknowledged and addressed, with respect and empathy. The NURS schema is helpful here. At the relationship level, the patient's desire for hope and help should be met with competence, honesty and commitment, while respecting the patient's autonomy. Forming a relationship begins before the actual consultation begins.

Specific clinical skills

Breaking bad news

Doctors are often confronted with situations where they must give the patient bad news.
This may be a cancer diagnosis, intrauterine death or a fetal condition, a negative result from infertility testing or treatment or in general, complications or unforeseen treatment outcomes.

When preparing for such conversations, certain fundamental rules must be followed. These help to remain empathetic and respectful of the patient in their situation. Important guiding principles are:

- Share the news in a quiet, closed space (no people walking through)
- Schedule enough time (at least 15 minutes)
- Ensure as few disruptions as possible. Switch off beepers and telephones or give them to someone else
- Before the conversation, the doctor should inform all those taking part in the conversation

When starting the conversation, after establishing an emotional connection with the patient (Joining: for example with a greeting "Hello [name], thank you for coming."), the patient's prior knowledge should be established: "What did you think when you felt the lump in your breast?", "How do you feel about your situation after chemotherapy?"

The words chosen for the conversation should explain the diagnosis, prognosis and other diagnostic or therapeutic steps simply and truthfully in short, clear sentences. Cancer, miscarriage, precancerous stages, death etc. should be talked about - the diagnosis should not be hidden behind vague euphemisms.

It is important to tell the patient they are about to receive bad news and to prepare them; e. g. "I'm afraid I don't have good news for you." This gives the patient an opportunity to prepare mentally.
After the diagnosis is given, the patient may need the doctor to give a prognosis. A different time should be given for a prognosis of how much time is left. Give the patient hope, but not false hope. "As you've asked me so openly about how long you have before you die, I'd like to answer openly too. I think you have a few, or maybe many, months left. We will try to support and help you as much as possible." The conversation should cover treatment possibilities as well as the diagnosis and prognosis. Of course, it is important to state that these are suggestions, and that the patient is involved in the decision-making process.

Sharing bad news is an exceptional emotional situation for both the patient and the doctor. The patient should be encouraged to express their thoughts and feelings. Waiting in silence after breaking bad news offers the patient room to acknowledge the information and react emotionally. Patients react differently to bad news: scared, paralyzed, confused, shocked, distressed, stoic, weeping, etc. All these emotions are normal reactions to being confronted with bad news.
As a first step, the patient's emotions should be acknowledged (emotional level) with sensitive verbal ("I can understand your reaction", "I understand you're very angry the lump wasn't discovered six months ago") or non-verbal (e. g. nodding, facing the patient and leaning towards them) reactions.

To conclude such conversations, the information discussed and the patient's reactions should be documented carefully, as well as the names of other informed persons. The patient should be offered an opportunity to talk about the situation again. Determine availability and responsibility, and highlight any other possibilities

for support (e. g. breast care nurse for breast cancer patients, midwives, psychologists, social services, etc.). In addition, it can be helpful to discuss with the patient which people in their entourage should be informed and involved.

SPIKES strategy

The SPIKES strategy from Buckman can be used as a reminder for how to prepare for breaking bad news:

- S: Setting up
- P: Perception
- I: Invitation
- K: Knowledge
- E: Emotions
- S: Summary and Strategy
  [B04]

- "Setting Up the Interview": Ensure a quiet, protected environment.
- "Assessing the Patient's Perception": Open questions to assess the patient’s perception: "What do you currently know about your medical situation?"
- "Obtaining the Patient's Invitation": Assess the patient’s readiness to receive bad news: "How should I present the test results?" Explain that it is possible to talk at a later point.
- "Giving Knowledge and Information to the Patient": Signaling before breaking bad news: "I'm sorry, I have bad news. Unfortunately, I have to tell you that..."
- Improve communication by: Adjusting to the patient’s vocabulary, i. e. using "non-technical words" (e. g. "spread" instead of "metastasized"). Share information in small units.
- "Addressing The Patient's Emotions With Empathic Responses": Identify and name emotions (e.g. sadness, anger, shock). Provide space to express and address feelings.
- "Strategy and Summary": Future steps are discussed, depending on the patient’s condition

Summary: Breaking bad news

Giving bad news is a conversation in an exceptional emotional situation, for both the patient and the doctor. The conversation should take place in a respectful environment. Certain basic rules governing the preparation of the conversation, how the conversation goes, the chosen language and how to proceed after the conversation must be followed. To prepare for the conversation, it is essential that a quiet room with as few interruptions as possible be chosen. Simple language should be used for the conversation, the diagnosis should be identified clearly by name and the patient should be prepared for the bad news before it is given.

Patients react to bad news in very different ways. The patient should be given space to express their emotions. The doctor should acknowledge emotions and react appropriately. If a prognosis is given, the patient should be given an honest answer, as well as hope and support.

Before the conversation is concluded, ensure that the patient is acknowledged and supported in their exceptional emotional situation. Establish whether or not relatives should be informed, if additional support could be helpful, how available the doctor can be if the patient has any questions, and when the next conversation is scheduled. The SPIKES strategy is a useful tool.

Risk consulting

Doctors must frequently inform patients of risks, for example in menopause consultations, oncology consultations, with contraception, before a surgery, etc.

The responsibilities of the doctor are to:

- Understand and evaluate the information
- Translating the information into patient-friendly language
- Give the patient a foundation for making health-related decisions

Patients often struggle to understand statistical data. Unclear statements on statistical data can lead to decisions being made on the basis of misinterpreted statistics. Typical errors are: Overwhelming use of relative risk, conditional probabilities, single event probabilities and framing (see below).

The article “Comprehensible risk communication made easy - or: How to avoid confusing probability statements” gives a good overview of the complex topic of risk consulting. [B14]

Framing

Framing means giving information context. In risk communication this means that the reference standard that applies to statistical data is relevant for the statement or its understanding. For example, we can talk about a 3 % risk of death or a 97 % chance of survival. Survival implies a positive outcome. Risk of death implies the risk of a negative outcome. The statements are based on the same statistical fact, but the emotional reaction to the information differs. To counter this, it is important to talk about chances, not just risks. The patient should be shown both perspectives.

- "The risk of having a child with Down syndrome is 1 in 250 for women of your age (0,5 %)."
- "The chances of having a child with no chromosome issues are 99 % for women of your age, 249 of every 150 children born have no chromosomal abnormalities."

Summary: Risk consulting

It is the doctor’s responsibility to make complex statistical information, i.e. the probability of a specific risk, understandable for the patient. It is helpful to use easily understandable figures to demonstrate verbal risk statements, e.g. using natural frequency or absolute risk. The reference scales used should change from the reference standard as little as possible and be explained in an easily understandable everyday perspective. Positive and negative outcomes should be identified. Risks should be visualized wherever possible. With risk consulting, it is important to take into account the patient’s value system and preferences, using shared decision making as explained in the next section.

Shared Decision Making

With shared decision making, the doctor and patient work together to make joint decisions on appropriate medical treatment.
This is done to ensure that the patient’s value system and preferences are taken into account in the decision. The role of the doctor is to provide evidence-based information on the risks and benefits of specific treatments in an understandable way. The patient’s role is to assign this information a level of importance i.e. apply their values and preferences.

Example of shared decision making: Menopause consultation

The following shows the key steps of participative decision making using the example of a 52-year-old female patient who has presented with disrupted sleep and hot flushes.

Example of shared decision making: Menopause consultation

Step 1

Clarification of the problem
Your disrupted sleep, which has a negative impact on your work, may be connected to the hot flushes and night sweats.

Evidence-based recommendation
The decision to take hormonal treatment or wait and see if things get better on their own must be made on an individual basis. Scientific studies show that hormones have both risks and benefits. In principle, we use the lowest effective dose, but there is a level of uncertainty when it comes to the risks.

Role clarification
Your opinions and assessment are important to me in deciding whether the benefits outweigh the risks for you.

Step 2

Hormones are very effective against hot flushes; if 1000 women with this symptom were taking hormones, 800 of them would be helped. There would be no or minimal effect for 200 of them. You would know within a few days if they help you.

Currently, the risks are these: Of those 1000 women we talked about, 999 would have no dangerous side effects. 1 in 1000 women will have a dangerous side effect such as thrombosis, heart attack, stroke, and if the hormones are taken for over 4 years breast cancer. At lose doses and with specific compositions, the risks may be lower.

Clarify the patient's values
What is more important to you right now: reducing the symptoms or worries about risks?

Screening for decision-making difficulties and help
Would you like more information?
Would you like to think about it and make an appointment for another discussion?

Figure 4: Example of shared decision making: Menopause consultation.

Summary: Shared Decision Making

Shared decision making aims to reach decisions made jointly by the patient and doctor. In this concept, communication is two-sided: Both parties communicate openly, information is exchanged, questions are asked and answered.

Advice on coping strategies

Advice on coping strategies generally follows a diagnosis or bad news such as cancer, chronic diseases such as endometriosis, surgical procedures with organ removal, changes to body image or after complications and/or unsuccessful pregnancy (miscarriage, premature birth etc.). Both of the following steps are necessary:

Establishing the problem

Open questions should be used to invite the patient to talk about their experiences, needs and challenges e.g.:

- "How are you now, two weeks after the surgery?"
- "When things aren't going well, how do you express that?"
- "How do you know that you're not yet an old woman?"
- "What sometimes helps you feel better?"
- "How have you reacted to the situation so far?"

The doctor can use empathetic statements to build a relationship with the patient and meet them on an emotional level (Patient-focused communication style).

- "Hearing about everything that's happened since the surgery, I can understand that you sometimes feel desperate."

Parts of the conversation should always be summarized in useful sections in order to ensure that the conversation is understandable for the patient and whether there are any further questions the patient would like to ask.

Understanding current coping strategies

"Coping" here means how the patient deals with their change in situation. We differentiate between emotional, cognitive and action-related coping strategies. These coping strategies can be more or less helpful depending on the individual situation:

Emotional coping strategies

- Struggling (self-pity, feeling the situation is unfair), resignation (fatalism, giving up hope), blaming others and expressing anger (either current with regard to

treatment or in relation to the past), blaming oneself and feelings of guilt.
- Repressing emotions rather than admitting them.
- Emotional relief, optimism, confident attitude to understanding of the disease.

Cognitive coping strategies

- Deflection, aggravation, problem analysis.
- Humor, irony, sarcasm
- Repression, denial, trivialization of the current burden
- Acceptance, putting into perspective versus others, giving meaning, turning to religion.

Action-related coping strategies

- Active avoidance, compensation, altruism, social withdrawal, tackling routine work, relaxation techniques, meditation, yoga, searching for information, making use of help from doctors and demanding attention.

The doctor can be supportive and therapeutically effective on each level.

On the emotional level, the patient can be supported with help to ease the burden, listening and understanding.

On the cognitive level, the identification and processing of “dysfunctional thoughts” can help. This refers to destructive thoughts. The patient and doctor can assess the truthfulness of thoughts together. For example, if the patient says “I'm definitely going to die”, challenge this and if necessary, change it to “I'm going to die one day. What support do I need to have the highest possible quality of life at this stage?”. A crisis should be seen as a challenge - an opportunity to become stronger. Support the patient in finding meaning in overcoming the challenge and concentrating on changing their life after the crisis.

On the action-related level, the doctor can help to identify helpful behaviors. This can be made accessible to the patient by asking questions, e.g. “Over the last 48 hours, when did you feel a little better?” and “What was different?”. Or: “In similar situations in your life, was there anything that helped you?” The doctor can encourage social support and decide with the patient which people in their entourage can help with specific needs. This can help the patient take control, decide what can and cannot be changed and make use of offers of help.

Summary: Advice on coping strategies

When giving advice on coping strategies, it is important to establish the patient's issue and classify and understand their current coping strategies. There are three levels of coping: emotional, cognitive and/or action-related. Coping strategies can be analyzed with the patient and classified as helpful or dysfunctional. Subsequent conversations can then attempt to work towards more constructive coping strategies.

Motivational Interviewing [B20]

In many clinical situations, the patient must change a specific health or disease-related behavior. But often, this change is not achieved. This applies above all to healthy habits and prevention, such as precautionary and early identification consultations, health advice, consultations with adolescents, menopause consultations, prenatal care and sex counseling.

According the behavioral model developed by Prochaska und Di Clemente (1983), when changing a behavior people go through different phases: [B19]

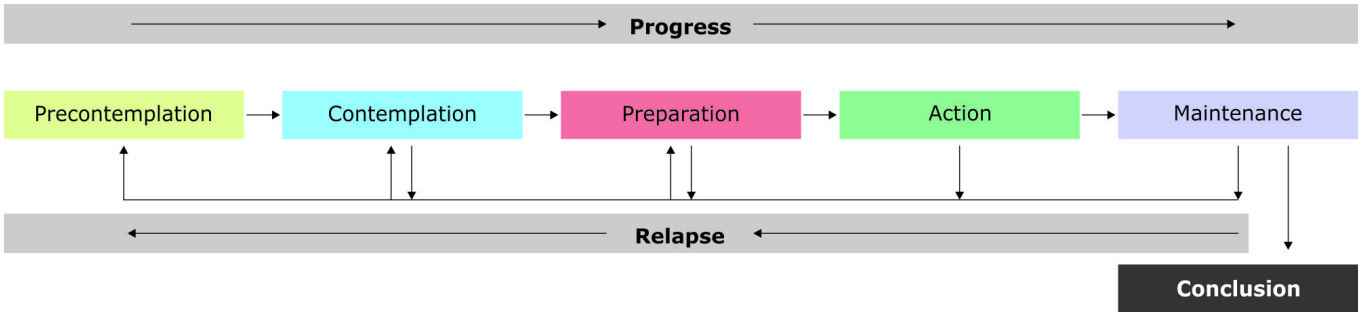

Figure 5: Behavior and behavioral change [B19]

By repeating these phases multiple times, new behavior can be practiced and developed into a new habit.

A major element of Motivational Interviewing is identifying the phase in which the patient is currently found, as various forms of communication are required. It is therefore particularly important to identify readiness for change during the interview and support the change process with the right interventions in the right phases.

Readiness for Change

There are two key questions for establishing readiness for change:

1. Identifying the importance of a change: "Why change?" This includes personal values, information and expectations regarding the importance of the change.
2. Identifying confidence in the patient's ability to make the change: "How to

change?” (perceptions of self-efficacy).

In a Motivational Interview, the doctor attempts to establish these factors with questions using scales:

- Importance: "On a scale of 0 to 10, how important is it for you to change your current behavior (0 = not important at all and 10 = extremely important)?"
- Confidence: "If you made the decision to change right now, how confident are you on a scale of 0 to 10 that you would reach your goal (0 = not confident at all and 10 = maximum confidence)?"
- Readiness: "How ready are you to change on a scale of 0 (not ready at all) to 10 (completely ready)?"

Carrying out a Motivational Interview

During a motivational interview, these factors (importance, confidence and readiness) can be covered in detail. As part of the discussion with the doctor, pros and cons of the change can be explored, and short and long-term consequences considered. Doubts and concerns about the current behavior can be addressed and the change anticipated. “Let’s imagine you've changed this behavior. How do you feel? How does your routine look?”
The doctor can help to structure the change in behavior and focus on small, manageable steps (from easy to hard) with a realistic timeframe. Overall targets can be distinguished from strategies and specific tasks. This means that the patient’s motivation to change their behavior can be continuously supported and reinforced. It is important to take into account previous successes and failures when changing behavior: “What has helped in the past? Where do you encounter difficulties?”. In case of relapse, it is important to focus on what can be learned “What went wrong? “, “What can we learn from this relapse? “.

A meta-analysis has been carried out to investigate the efficacy of motivational interviewing in various problem areas. It was shown to have a moderate effect. [B05]

Summary: Motivational Interviewing

In clinical situations where a behavioral change is the objective, motivational interviewing can be beneficial. It is important to establish which stage of making a change the patient is currently in so that suitable conversation techniques can be used. Establishing readiness for change is essential. Other key elements are the importance of the change and confidence in the ability to make the change. Supporting a patient can mean discussing advantages, disadvantages, possible consequences and difficulties together. Successes and failures can be discussed openly in order to encourage the patient’s motivation to change their behavior.

Literature

Nr.	Title	Author	Journal	Volur
[B01]	Die Arzt-Aerztin-Patientin Kommunikation in der Konsultation - Grundlagen, Techniken, Schwierigkeiten und Lösungsmöglichkeiten	Bitzer J (Hrsg: Bodden-Heidrich R, Rechenberger I, Bender HG)	Psychosomatische Gynäkologie und Geburtshilfe, Psychosozial Verlag Giessen	
[B02]	Psychosoziale und psychosomatische Basiskompetenz des Frauenarztes – von der inneren Überzeugung zum lernbaren Curriculum.1. Kommunikation.	Bitzer J, Tschudin S, Schwendke A, Alder J	Gynäkologisch-geburtshifliche Rundschau	41
[B03]	Psychosoziale und psychosomatische Basiskompetenz des Frauenarztes von der inneren Überzeugung zum lernbaren Curriculum. 2. Biopsychosoziales Modell	Bitzer J, Tschudin S, Schwendke A, Alder J	Gynäkologisch-geburtshilfliche Rundschau	42
[B04]	Breaking bad news: the S-P-I-K-E-S strategy	Buckman R	Psychosocial Oncology	2
[B05]	The Efficacy of Motivational Interviewing: A Meta-Analysis of Controlled Clinical Trials	Burke BI, Arkowitz H, Menchola M	J Consult Clin Psychol	71
[B06]	The skilles helper: A problem management approach to helping	Egan G	Brooks/Cole. Pcafic Grove	

Nr.	Title	Author	Journal	Volur
[B07]	The Need for a New Medical Model: A Challenge for Biomedical Science	Engel GL	Science 196	129
[B08]	Communication evidence for participatory decision making	Epstein RM, Alper BS, Quill TE	JAMA	291
[B09]	Giving sad and bad news	Followfield L	Lancet	341
[B10]	The Medical Interview	Lipkin M, Putnam SM, Lazare A (eds)	Springer New York Berlin Heidelberg	
[B11]	Calculated risks-How to know when numbers deceive you	Gigerenzer G	Simon and Schuster New York	
[B12]	Helping Doctors and Patients make Sense of Health Statistics	Gigerenzer G, Gaissmaier W, Kurz-Milcke E, Schwartz LM, Woloshin S	Ass Psychol Science	8(2)
[B13]	A randomized clinical trial for women with vulvodynia: Cognitive-behavioral therapy vs. supportive psychotherapy	Masheb RM, Kern RD et al.	Pain	141
[B14]	Verständliche Risikokommunikation leichtgemacht oder: wie man verwirrende Wahrscheinlichkeitsangaben vermeidet 2005	Mata J, Dieckmann A, Gigerenzer G	Georg Thieme Verlag KG Stuttgart	81

Nr.	Title	Author	Journal	Volur
[B15]	Patient Centeredness: A conceptual framework and review of the empirical literature	Mead N, Bower P	Social Science and Medicine	51
[B16]	Patient-Centered Care and Preference-Sensitive Decision Making	Keirns C, Goold SD.	JAMA	302(1
[B17]	Arzt-Patient-Kommunikation. Mitteilen schlechter Nachrichten	Langewitz W	In: Brähler S., Strauss B. (eds) Lehrbuch der medizinischen Psychologie und Soziologie. Hofgrefe	
[B18]	Risk communication in practice: the contribution of decision aids	O'Connor AM, Légaré F, Stacey D.	BMJ	327
[B19]	Stages and Processes of Self-Change of Smoking: Toward An Integrative Model of Change	Prochaska JO, DiClimente CC.	Journal of Consulting and Clinical Psychology	51
[B20]	Health Behavior Change. Churchill Livingstone	Rollnick S, Mason P, Butler C (eds)	Edingburgh, London	
[B21]	Gelingende Kommunikation - revisited. Ein Leitfaden für partnerorientierte Gesprächsführung, wertschöpfende Verhandlungsführung und lösungsfokussierte Konfliktbearbeitung	Rosner S	Mering	3

Nr.	Title	Author	Journal	Volur
[B22]	Patient-centered medicine. Transforming the clinical method	Stewart M, Stewart M, Belle Brown J et al.	Sage, Thousand Oaks	
[B23]	Group coping skills instruction and supportive group psychotherapy for cancer patients – a comparison of strategies	Telch CF, Telch MJ.	J Consult Clin Psychol	54
[B24]	Was heisst Psychosomatik	von Uexküll T	Schweiz. Med. Wschr	114
[B25]	Integrierte Medizin	von Uexküll T, Geigges W, Plassmann R	Schattauer Stuttgart	
[B26]	WHO Collaborative Study: an epidemiological survey of psychological problems in general health care in 15 centers worldwide	Ustün BT	Int J Psychiatry	6